

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: EGPatient ID: EGPeriod: March 15 to May 6, 2024Report Date: May 7, 2024Coverage: Bed: 915/1245h (73.5%)Chair: 670/1245h (53.8%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 53 days from Mar 15 to May 06

HRBreathing

Elevated Breathing Mar 27 to Apr 04 (9d)	Elevated Breathing Apr 11 to Apr 17 (7d)	Elevated Breathing Apr 20 to Apr 26 (7d)	Elevated Breathing May 05 to May 05 (1d)	High Breathing Apr 25 to Apr 27 (3d)	High Breathing May 05 to May 05 (1d)
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Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	60 bpm	48 bpm	79 bpm
Breathing	18 brpm	11 brpm	46 brpm

Last 24 hours: one very high breathing episode in the afternoon; one elevated breathing episode in the afternoon; one high breathing episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 05	1 PM to 2 PM	3h	3	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	7 PM to 8 PM	1h	1	High Breathing (brief)	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	4 PM to 5 PM	1h	1	Very High Breathing (brief)	42	42	42	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

39 episodic events Detected, spanning 45h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate, Very High Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
39	45h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	4 PM to 5 PM	2h	2	High Breathing	35	23	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	7 PM to 8 PM	1h	1	High Breathing	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 27	12 PM to 2 PM	14h	10	Elevated Breathing	27	19	40	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 11	2 PM to 3 PM	14h	12	Elevated Breathing	26	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 20	6 PM to 7 PM	7h	7	Elevated Breathing	27	16	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	1 PM to 2 PM	2h	2	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

RR values outside physiologic range (6–60 breaths/min).

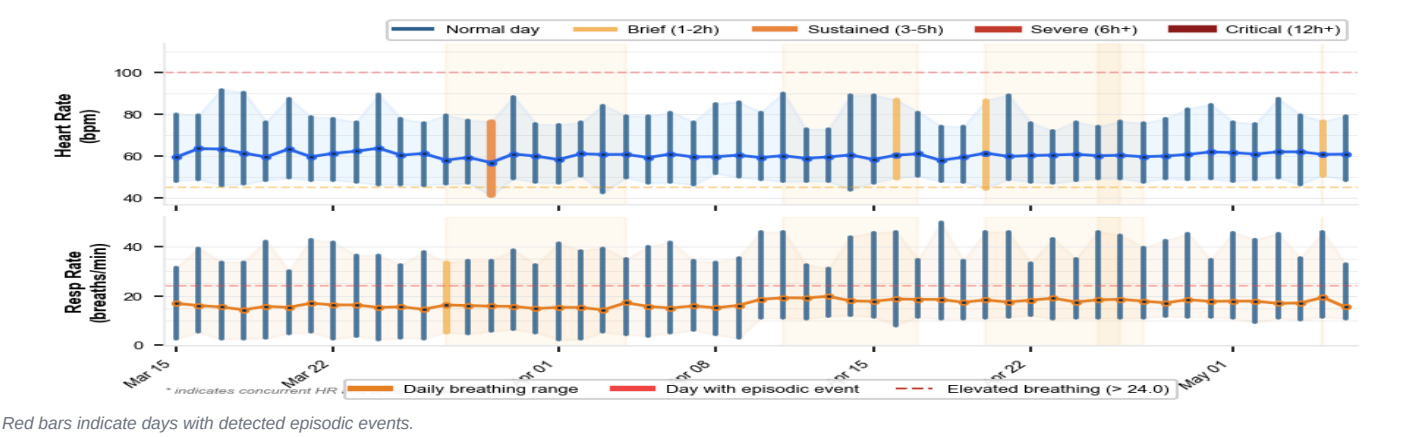
Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 40 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

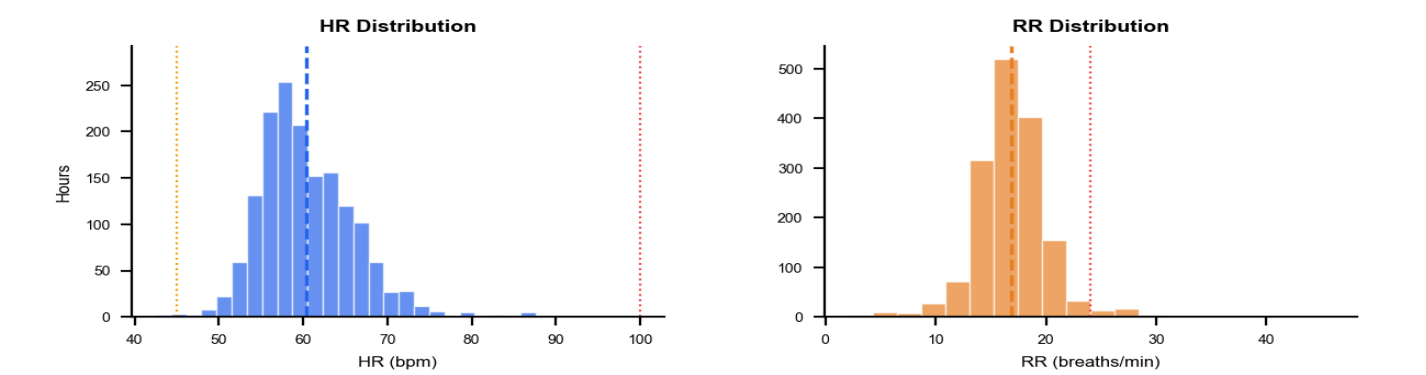
- Very High Breathing (avg 44 bpm, peak 46 bpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- High Breathing (avg 35 bpm, peak 46 bpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

Clinical Pattern Observations:

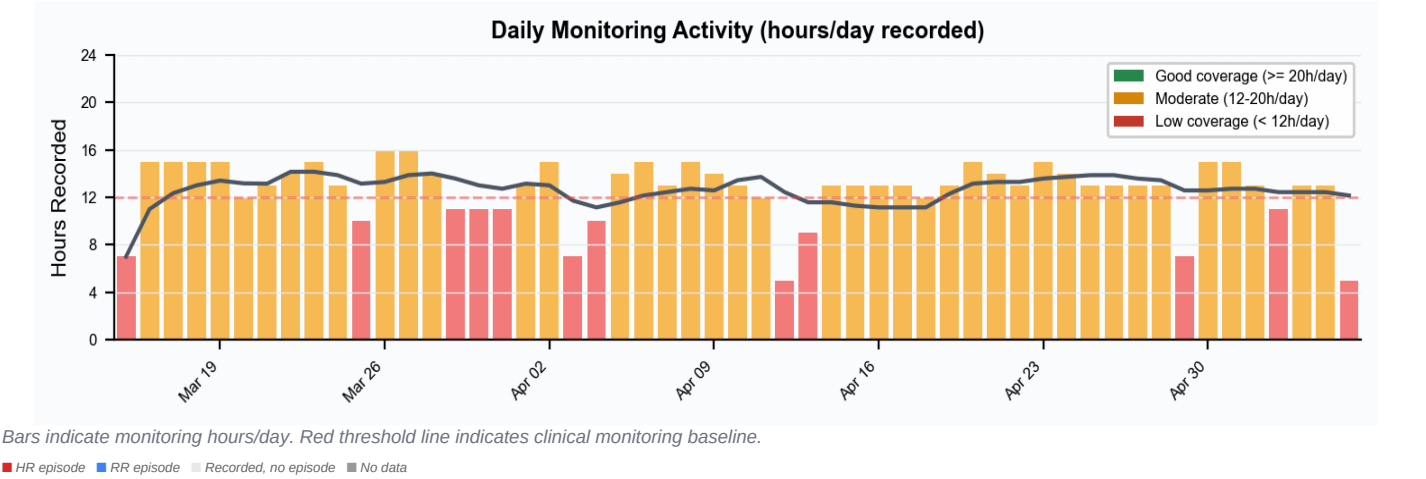
- 1. **Clustered pattern:** Episodic events on 5 of 53 days (9%).
- 2. **Daytime pattern:** Episodes concentrated during waking hours.



Vital Sign Distribution



Monitoring Activity



Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.